Item 1: Random
sequence generation

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sequence generation

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sequence generation

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sequence generation
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sequence generation

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sequence generation

Item 2: Allocation
concealment

Item 2: Allocation
concealment

Item 2: Allocation concealment
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Item 2: Allocation concealment

Item 2: Allocation concealment

Item 2: Allocation concealment
Item 2: Allocation concealment
Item 3: Blinding of participants

Item 3: Blinding of participants

Item 3: Blinding of participants
Item 3: Blinding of participants

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Item 3: Blinding of participants

Item 3: Blinding of participants

Item 3: Blinding of participants
Item 4: Blinding of healthcare providers

Item 4: Blinding of healthcare providers

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Item 4: Blinding of healthcare providers

Item 5: Blinding of outcome assessors

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Step 1: Was the allocation sequence adequately generated

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Step 2: Judge risk of bias related to sequence generation

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Step 2: Judge risk of bias related to sequence generation

Step 1: Was the allocation adequately concealed

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Step 2: Judge risk of bias related to allocation concealment

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Step 2: Judge risk of bias related to allocation concealment

Step 1: Were participants blinded

Step 1: Were participants blinded

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Step 2: Judge risk of bias related to blinding of participants(Issues to consider:
Were participants blinded (step 1)
If unblinded, how likely unblinding of participants has influenced the outcome
➢ How likely are participants expectations regarding effect of intervention to have influenced the outcome
➢ How likely are participant-initiated co-interventions to have influenced the outcome)

Step 2: Judge risk of bias related to blinding of participants(Issues to consider:
Were participants blinded (step 1)
If unblinded, how likely unblinding of participants has influenced the outcome
➢ How likely are participants expectations regarding effect of intervention to have influenced the outcome
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Step 1: Were healthcare providers blinded

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If unblinded, how likely unblinding of healthcare providers has influenced the outcome
➢ How likely healthcare provider-initiated co-interventions have influenced the outcome)

Step 1: Were outcome assessors blinded (When the outcome is participants self-report, e.g., completing a questionnaire (such as quality of life, disability index), participants are the outcome assessors.)

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Step 2: Judge risk of bias related to blinding of outcome assessors(Issues to consider:
Were outcome assessors blinded (step 1)
If unblinded, how likely unblinding of outcome assessors has influenced the outcome assessment
➢ Degree of judgement/subjectivity involved in the outcome assessment (more judgment/subjectivity more likelihood of bias))
Step 2: Judge risk of bias related to blinding of outcome assessors(Issues to consider:
Were outcome assessors blinded (step 1)
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Step 2: Judge risk of bias related to blinding of outcome assessors(Issues to consider:

Were outcome assessors blinded (step 1)

If unblinded, how likely unblinding of outcome assessors has influenced the outcome assessment

➢ Degree of judgement/subjectivity involved in the outcome assessment (more judgment/subjectivity more likelihood of bias))

Definitely Yes

Probably Yes

Probably No

Definitely No

Definitely Yes

Probably Yes

Probably No

Definitely No

Definitely Yes

Probably Yes

Probably No

Definitely No

Definitely Yes

Probably Yes

Probably No

Definitely No

Definitely Yes

Probably Yes

Probably No

Definitely No

Definitely Yes

Probably Yes

Probably No

Definitely No

Definitely Yes

Probably Yes

Probably No

Definitely No

Definitely Yes

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Probably No

Definitely No

Definitely Yes

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Probably No

Definitely No

Definitely Yes

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Probably No

Definitely No

Trial explicitly stated use of an adequate method of generating the random allocation sequence. Examples include: random number table; random number generator; throwing dice; drawing of
Trial was described as “randomized” without further details regarding the method of generating the allocation
sequence, and:
the text mentioned simple randomization, block randomization, or stratified randomization; or
the text described the allocation concealment method (stated using central allocation, drug
Trial was described as “randomized” without further details regarding the method of generating the allocation
Trial used a non-randomized allocation sequence that would be recognized as “quasi-randomization”. Examples
include: allocation based on dates of birth or admission; patient’s hospital record number; alteration or rotation;
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include: allocation based on dates of birth or admission; patient’s hospital record number; alteration or rotation;
Trial used a clearly satisfactory allocation concealment method: central allocation (e.g., telephone, web-based);
pharmacy-controlled randomization (stated using sequentially numbered sealed drug
● Trial explicitly stated using sequentially numbered, opaque, sealed envelopes without further details.
● Drug trial in which participants and healthcare providers were blinded: -
with no further information on the allocation concealment method; or -
stated using drug containers but it remains unclear whether the drug containers were

● Unblinded drug trial or non-drug trial: -
with no further information on the allocation concealment method; or - -
stated using envelopes but it remains unclear whether the envelopes were sequentially numbered opaque
and sealed; or
stated using drug containers but it remains unclear whether the drug containers were
● Trial used an open random allocation schedule.
● Trial used a non-randomized allocation sequence that would be recognized as “quasi-
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● Trial used a non-randomized allocation sequence that would be recognized as “quasi-

Trial explicitly stated that participants were blinded.
No explicit statement about blinding of participants, and: - it is a placebo-controlled drug trial; or - it is an active control drug trial (A vs. B) and mention of “double dummy” or that medications were identical or matched; or - trial was described as “single blinded”, “double blinded” or “triple blinded”, and the best judgment is that one of the blinded groups is the
No explicit statement about blinding of participants, and: - it is an active control drug trial (A vs. B) and no mention of “double dummy” or that medications were identical or matched; or - it is a non-drug trial; or - trial was described as “single blinded” and the best judgement is that the
● Trial explicitly stated that participants were not blinded.
● Trial was described as “open-label” or “unblinded”.

● Participants were definitely blinded; OR
● Unblinding of participants very unlikely to have influenced the outcome because: very unlikely participants expectations regarding effect of
intervention have influenced the outcome and very unlikely participant-initiated co-interventions have influenced the outcome.

● Participants were probably blinded; OR
● Unblinding of participants unlikely to have influenced the outcome because: unlikely participants expectations regarding effect of
intervention have influenced the outcome and unlikely participant-initiated co-interventions have influenced the outcome.

Participants were definitely or probably not blinded, AND unblinding of participants likely to have influenced the outcome because participants
expectations regarding effect of intervention likely to have influenced the outcome or participant-initiated co-interventions likely to have
influenced the outcome.

Participants were definitely or probably not blinded, AND unblinding of participants very likely to have influenced the outcome through
participants expectations regarding effect of intervention or through participant-initiated co-interventions.

Trial explicitly stated that healthcare providers were blinded.
No explicit statement about blinding of healthcare providers, and: - - -
it is a placebo-controlled drug trial; or
it is an active control drug trial (A vs. B) and mention of “double dummy” or that medications were identical or matched; or
trial was described as “single blinded”, “double blinded” or “triple blinded”, and the best

No explicit statement about blinding of healthcare providers, and: - - -
it is an active control drug trial (A vs. B) but no mention of “double dummy” or that medications were identical or matched; or
it is a non-drug trial; or
trial was described as “single blinded” and the best judgement is that the single blinded group is

● Trial explicitly stated that healthcare providers were not blinded.
● Trial was described as “open-label” or “unblinded”.

Healthcare providers were definitely blinded.

● Healthcare providers were probably blinded; OR
● Unblinding of healthcare providers unlikely to have influenced the outcome because: - Investigators have documented all healthcare provider-initiated co-interventions that could potentially influence the outcome and
demonstrated similarity in use of all these co-interventions between groups.

● Healthcare providers were definitely or probably not blinded, AND unblinding of healthcare providers likely to have influenced the outcome
because there are healthcare provider-initiated co-interventions that could potentially influence the outcome.

● Healthcare providers were definitely or probably not blinded, AND unblinding of healthcare providers very likely to have influenced the
outcome because there are healthcare provider-initiated co-interventions that could potentially influence the outcome and investigators
have documented dissimilarity in any of these co-interventions between groups.

Trial explicitly stated that the outcome assessors or adjudicators (people making the measurement or assessment) were blinded.
No explicit statement about blinding of outcome assessors or adjudicators, and: - - -
it is a placebo-controlled drug trial; or
it is an active control drug trial (A vs. B) and mention of “double dummy” or that medications were identical or matched; or
trial was described as “single blinded”, “double blinded” or “triple blinded”, and the best
No explicit statement about blinding of outcome assessors or adjudicators, and: - - -
it is an active control drug trial (A vs. B) but no mention of “double dummy” or that medications were identical or matched; or
it is a non-drug trial; or
trial was described as “single blinded” and the best judgement is that the single blinded group is

- Trial explicitly stated that the outcome assessors or adjudicators were not blinded.
- Trial was described as “open-label” or “unblinded”.

- Outcome assessors were definitely blinded; OR
- Outcome is all-cause mortality.

- Outcome assessors were probably blinded; OR
- Unblinding of outcome assessors likely could not have influenced the outcome assessment because the outcome assessment involves
minimal judgement (other objective outcomes, e.g., laboratory measurement, hospital admission, mechanical ventilation).

Outcome assessors were definitely or probably not blinded, AND unblinding of outcome assessors likely could have influenced the outcome
assessment because the outcome assessment involves some judgment (e.g., cause-specific mortality).

Outcome assessors were definitely or probably not blinded, AND unblinding of outcome assessors could have influenced the outcome
assessment because the outcome assessment involves considerable judgment by participant or adjudicator (e.g., symptoms and symptom
scores, quality of life, seizure occurrence).